

=====

SECTION 1: FAIR BALANCE REQUIREMENT

=====

Under 21 CFR 202.1(e)(5), all prescription drug advertisements must present a fair balance of information about the risks and benefits of the advertised drug.

Fair Balance Requirements:

- The presentation of risk information must be comparable in scope, depth, and prominence to the presentation of benefit information.
- Risk information must NOT be minimized, overshadowed, or presented in a manner that discourages a patient from reading or listening to it.
- Audio presentations of risk information must use the same speaking rate, volume, and clarity as benefit presentations.
- Visual presentations must not diminish risk disclosures through distracting imagery, small font size, or rapid scrolling.
- Adequate time/space must be devoted to risk information.

Violations include:

- Presenting benefits for 60 seconds and risks for 5 seconds.
- Using upbeat music over the risk disclosure section.
- Showing happy, active people during the risk narration (visual-audio mismatch).
- Listing only mild side effects while omitting serious or life-threatening risks.

=====

SECTION 2: REQUIRED DISCLOSURES

=====

Under 21 CFR 202.1(e)(1), a prescription drug advertisement must include:

1. THE ESTABLISHED (GENERIC) NAME of the drug – must appear prominently and immediately adjacent to the most prominent display of the proprietary (brand) name.
The established name must be in type at least half as large as the proprietary name.
2. QUANTITATIVE COMPOSITION – the active ingredients.
3. BRIEF SUMMARY – a concise statement of the drug's side effects, contraindications, and effectiveness. For broadcast ads, a major statement of the most important risks must be included with direction to access the full prescribing information.
4. ADEQUATE PROVISION – for broadcast ads, must provide:
 - Toll-free phone number for full prescribing information
 - Website URL
 - Reference to healthcare provider consultation
 - Statement to see the ad in a publication
5. DRUG CATEGORY – the drug's pharmacological class or principal mode of action.

Missing any of the above constitutes a CRITICAL violation.

=====

SECTION 3: PROHIBITED CLAIMS AND LANGUAGE

=====

The following language is PROHIBITED or RESTRICTED in drug advertising:

Absolutely Prohibited:

- "Cure" – unless the drug is FDA-approved to cure the condition
- "Guarantee" or "guaranteed results" – no drug outcomes can be guaranteed
- "100% safe" / "completely safe" / "no side effects" – all approved drugs carry risks
- "Miracle drug" / "miracle treatment"
- "Risk-free" – all prescription drugs carry risks
- "Best drug" / "most effective" – requires head-to-head comparative clinical data

Restricted Language Requiring Clinical Substantiation:

- Any superlative claim (e.g., "fastest acting", "most powerful") requires substantial evidence from well-controlled clinical trials.
- Comparative claims vs. competitor drugs require head-to-head clinical studies with the specific comparator drug.

False or Misleading Statements [21 CFR 202.1(e)(6)]:

- Statements that create a misleading impression through suggestion, implication, or omission are violations even if literally true.
- Presenting only the best-case clinical results without context is misleading.
- Using patient testimonials that claim results not typical of all patients without clear disclosure.

=====

SECTION 4: UNSUBSTANTIATED EFFICACY CLAIMS

=====

Every efficacy claim in a drug advertisement must be:

1. Consistent with the FDA-approved labeling (Prescribing Information / Package Insert).
2. Supported by substantial evidence – defined as adequate and well-controlled clinical investigations as required under 21 CFR 314.126.
3. Not extrapolated beyond the approved indication.

Examples of Unsubstantiated Claims:

- "Shown to reduce symptoms by up to 90%" – requires citation of the specific trial.
- "Clinically proven" – must reference the specific studies.
- "Doctors recommend" – requires quantitative survey data.
- Claims of mechanism-of-action benefits not demonstrated in clinical outcomes.

Substantiation Requirements:

- Cite the clinical trial name, number of patients, duration, and comparator.
- Efficacy statistics must be from the approved drug label.
- Subgroup results must not be presented as results for the whole population.

=====

SECTION 5: VISUAL AND AUDIO STANDARDS

=====

Visual-Audio Consistency:

- Imagery must not contradict or diminish the audio risk disclosure.
- Example violation: showing children playing happily while audio states "may cause serious birth defects if taken during pregnancy."
- Example violation: upbeat background music during serious adverse event disclosure.
- Rapid visual cuts during risk disclosure make it difficult to process information.

Superimposed Text:

- If risk information is presented as superimposed text, it must be:
 - Legible (adequate font size, contrast, duration on screen)
 - Not obscured by background imagery
 - Consistent with the audio track

Patient Portrayals:

- Must not imply results typical of all patients if individual results vary.
- Must include "Individual results may vary" or equivalent when using testimonials.
- Must not show patients performing activities not demonstrated as safe.

SECTION 6: OFF-LABEL PROMOTION

21 CFR 202.1(e)(6)(ii) prohibits promoting drugs for uses not approved by FDA.

Off-Label Promotion includes:

- Mentioning unapproved indications (different diseases, age groups not in labeling).
- Using images implying uses beyond the approved indication.
- Discussing dosing regimens not in the approved label.
- Mentioning investigational (not yet approved) uses.

Exception: Scientific exchange and medical education materials are governed by separate FDA guidance (Good Reprint Practices, 2009 FDA Guidance).

SECTION 7: SPECIAL POPULATIONS

Advertisements targeting or implying use in special populations must comply with:

Pediatric Populations:

- Must not imply pediatric use unless the drug is FDA-approved for that age group.
- Showing children in the ad implies pediatric appropriateness.

Elderly Patients:

- Geriatric labeling requirements must be reflected in the ad.
- Dose adjustments in elderly populations must be disclosed if clinically significant.

Pregnant and Lactating Women:

- Pregnancy category or PLLR (Pregnancy and Lactation Labeling Rule) information must be accessible.
- Risks to fetus or infant must not be minimized.

SECTION 8: ENFORCEMENT AND PENALTIES

FDA Enforcement Tools:

- Untitled Letters (low-level violations)
- Warning Letters (serious violations – publicly posted)
- Injunctions
- Criminal prosecution for willful violations
- Civil money penalties

Civil Penalties: Up to \$250,000 per violation for individuals; \$1,000,000 for companies.

Post-Enforcement Corrective Advertising:

- FDA can require companies to run corrective advertisements to counteract false impressions created by violative ads (as in the Paxil case precedent).